

Official Title: A Phase II, Open-Label, Multicenter, Single-Arm Study to Investigate the Efficacy and Safety of Atezolizumab as Neoadjuvant and Adjuvant Therapy in Patients With Stage IB, II, IIIA, or Selected IIIB Resectable and Untreated Non-Small Cell Lung Cancer

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STATISTICAL ANALYSIS PLAN

Genentech, Inc.
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South San Francisco, CA 94080

**A PHASE II, OPEN-LABEL, MULTICENTER, SINGLE-ARM
STUDY TO INVESTIGATE THE EFFICACY AND SAFETY
OF ATEZOLIZUMAB AS NEOADJUVANT AND
ADJUVANT THERAPY IN PATIENTS WITH STAGE IB, II,
IIIA OR SELECTED IIIB RESECTABLE AND UNTREATED
NON-SMALL CELL LUNG CANCER**

Protocol No: ML39236 Version 5

Prepared by:


Syneos Health

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ATEZOLIZUMAB AS NEOADJUVANT AND ADJUVANT THERAPY
IN PATIENTS WITH STAGE IB, II, IIIA OR selected IIIB
RESECTABLE AND UNTREATED NON-SMALL CELL LUNG
CANCER**

Protocol No: ML39236 Version 5

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Genentech, Inc.

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LIST OF ABBREVIATIONS AND DEFINITION OF TERMS

Abbreviation	Definition
CNB	core needle biopsy
CR	complete response
CT	computed tomography
CTCAE	Common Terminology Criteria for Adverse Events
████	████████████████
ECOG	Eastern Cooperative Oncology Group
████	████████████████
eCRF	electronic Case Report Form
HBsAg	hepatitis B surface antigen
HBV	hepatitis B virus
HCV	hepatitis C virus
██	████████████████
IHC	immunohistochemistry
INR	International normalized ratio
IRB	Institutional Review Board
MRI	magnetic resonance imaging
NCI	National Cancer Institute
NSCLC	non–small cell lung cancer
ORR	objective response rate
OS	overall survival
PCR	polymerase chain reaction
PD-1	programmed death-1
PD-L1	programmed death-ligand 1
PET	positron emission tomography
PFT	pulmonary function test
q21d	every 21 days
q3w	every 3 weeks
RECIST	Response Evaluation Criteria in Solid Tumors
SOC	standard of care
UC	urothelial cancer
WES	whole-exome sequencing

1. INTRODUCTION

1.1 Background on Non-small Cell Lung Cancer

Lung cancer is the leading cause of cancer death worldwide. In the United States about 200,000 cases are diagnosed each year with approximately 150,000 deaths occurring and a progressive increase in mortality with age (Siegel et al. 2015). Non-small cell lung cancer (NSCLC) accounts for approximately 85% of all lung cancers and is divided into squamous cell carcinoma, large cell carcinoma, and adenocarcinoma subtypes. The majority of patients present with advanced metastatic disease that cannot be cured with current therapies. A minority of patients (about 25%) present with localized disease that is sometimes (about 50%) cured by surgical resection. Of those not cured by surgical resection, the majority recur in distant sites that could not be detected prior to surgical resection. For these Stage IB-IIIa patients, the addition of systemic chemotherapy before or after surgery improved 5-year survival rates by about 5% (Pignon et al. 2008). Chemotherapy before surgery (neoadjuvant) results in slightly higher response rates compared to chemotherapy for advanced stages but there are essentially no complete responses (CRs). Recently, immunotherapy with antibodies that bind to checkpoint inhibitors such as programmed death 1 (PD-1) and programmed death ligand (PD-L1) have been shown to produce long lasting responses in some patients with advanced NSCLC who were refractory to standard chemotherapy (Rizvi et al. 2015).

Subsequent randomized clinical trials showed that these antibodies produce superior survival compared to docetaxel chemotherapy in patients who progressed after systemic platinum doublet chemotherapy (Borghaei et al. 2015; Brahmer et al. 2015; Fehrenbacher et al. 2016). The clinical benefits were greater in patients with high PD-L1 expression although survival was similar to docetaxel even in patients lacking PD-L1 expression. Early studies with these antibodies in the first line therapy setting showed even better results especially in patients with high PD-L1 expression (Gettinger 2015; Garon et al. 2015). Because PD-L1 expression is a continuous variable and not a perfect predictive biomarker, many investigators are exploring other potential biomarkers of predictive therapeutic benefit. The number of mutations determined by whole-exome sequencing (WES) has been shown to perform as another potential predictive biomarker (Rizvi et al. 2015). The expression of many other genes and proteins is being explored, but this is complicated by the lack of tissue and a limited understanding of its association with objective response rates (ORR) and survival.

1.2 Background on Atezolizumab

PD-L1 is an extracellular protein that down regulates immune responses primarily in peripheral tissues through binding to its two receptors PD-1 and B7.1. PD-1 is an inhibitory receptor expressed on T cells following T-cell activation, which is sustained in states of chronic stimulation such as in chronic infection or cancer (Blank et al. 2005; Keir et al. 2008). Binding of PD-L1 with PD-1 inhibits T-cell proliferation, cytokine production, and cytolytic activity, leading to the functional inactivation or exhaustion of T cells. B7.1 is a molecule expressed on antigen-presenting cells and activated T cells. PD-L1 binding to B7.1 on T cells and antigen-presenting cells can mediate down regulation of immune responses, including inhibition of T-cell activation and cytokine production (Butte et al. 2007; Yang et al. 2011).

Overexpression of PD-L1 on tumor cells (TCs) and the tumor microenvironment has been reported to impede anti-tumor immunity, resulting in immune evasion (Blank and Mackensen 2007). Therefore, interruption of the PD-L1/PD-1 pathway represents an attractive strategy to reinvigorate tumor-specific T-cell immunity.

Atezolizumab (Tecentriq™, formerly MPDL3280A) is a humanized IgG1 monoclonal antibody consisting of two heavy chains (448 amino acids) and two light chains (214 amino acids), and is produced in Chinese hamster ovary cells. Atezolizumab was engineered to eliminate Fc-effector function via a single amino acid substitution at position 298 on the heavy chain, which results in a non-glycosylated antibody that has minimal binding to Fc receptors and prevents Fc-effector function at expected concentrations in humans. Atezolizumab targets human PD-L1 and inhibits its interaction with its receptors, PD-1 and B7.1 (CD80, B7-1). Both of these interactions are reported to provide inhibitory signals to T cells. Direct targeting of PD-L1 leaves the PD-L2-PD-1 interaction intact, potentially avoiding effects on immune homeostasis.

Atezolizumab is approved in the United States for the treatment of locally advanced or metastatic urothelial cancer (UC), and metastatic NSCLC that has progressed during or following treatment with a platinum-containing regimen. Atezolizumab is also being investigated as a potential therapy against solid tumors and hematologic malignancies in humans.

1.3 Study Rationale and Benefit-Risk Assessment

It is hypothesized that administration of atezolizumab, an engineered IgG1 monoclonal antibody targeting PD-L1, will demonstrate a good safety profile and produce objective responses in clinical Stage IB-III NSCLC, especially in tumors which express PD-L1 prior to resection. PD-L1 expression by tumor and immune cells results in impaired anti-tumor immune responses by inhibiting T-cell proliferation, cytokine production, and cytotoxic activity. As a result there is significant interest in developing therapeutics to block the immunosuppressive effects of PD-L1 and to identify patients who will benefit from this treatment strategy.

While the addition of adjuvant cisplatin based chemotherapy following surgical resection of early stage NSCLC has produced significant survival benefits (Pignon et al. 2008), rates of local and distant disease relapse leading to death remain unacceptably high. Studies show that neoadjuvant chemotherapy has approximately the same benefit as adjuvant (Felip et al. 2010). Thus, it is critical to investigate the use of atezolizumab and similar agents in treating early stage resectable disease where one can assess the immune environment before and after treatment. It is also critical to develop highly predictive biomarkers of benefit and to study the evolution of immune-related markers with treatment with atezolizumab.

There will be two parts to this study: the first part (Neoadjuvant Atezolizumab Therapy Phase) will evaluate the ability of atezolizumab, an engineered IgG1 monoclonal antibody targeting PD-L1, to produce pathologic responses in the neoadjuvant setting in patients with early stage NSCLC who have a pretreatment biopsy.

The primary endpoint of the study will be major pathologic response rate (defined as $\leq 10\%$ of viable tumor cells) determined from the surgical resection (Hellmann et al. 2014). In addition, response rates according to RECIST v1.1 will be determined from chest CT scans obtained before and after the atezolizumab therapy and before surgical resection.

The Neoadjuvant Atezolizumab Therapy Phase will only include patients with Stage IB, II, IIIA, or selected IIIB disease and who are deemed suitable for surgical resection without metastatic disease but with sufficient material for initial biopsy to analyze biomarkers. Clinical staging of NSCLC is based on computed tomography (CT) of the chest and upper abdomen, positron emission tomography (PET) and brain CT or magnetic resonance imaging (MRI) to rule out metastatic disease and assess the potential for curative-intent resection. Resection represents the best chance for prolonged survival for patients with non-advanced NSCLC.

There is little data to guide the duration of adjuvant immunotherapy in lung cancer. An ongoing Phase III study is evaluating 12 months of atezolizumab treatment after cisplatin-based chemotherapy in patients with completely resected Stage IB-IIIA NSCLC with high PD-L1 expression. Another Phase III study is currently recruiting patients with unresectable stage III NSCLC to receive 1 year of consolidation anti-PD-1 therapy after definitive chemo-radiation. Adjuvant immunotherapy has also been explored in other solid tumors. Adjuvant immunotherapy treatment for 1 year with interferon alfa-2b prolongs the relapse-free interval and OS of high-risk resected melanoma patients (Kirkwood et al. 1996). One year of adjuvant anti-PD-1 therapy is considered standard of care for resected melanoma based on the data from the CHECKMATE-238 study (Weber et al. 2017).

2. STUDY OBJECTIVES

This study will evaluate the efficacy and safety of atezolizumab in patients with NSCLC. Specific objectives and corresponding endpoints for the study are outlined in Table 1.

Table 1 Objectives and Corresponding Endpoints

Objectives	Corresponding Endpoints
Primary Efficacy Objective:	
To evaluate the efficacy of atezolizumab as neoadjuvant treatment for Stage IB, II, IIIA, and selected IIIB NSCLC	Major pathologic response (defined as $\leq$ 10% of viable tumor cells), scored by a pathologist, based on surgical resection as defined by prior studies (Hellmann et al. 2014)
Secondary Efficacy Objectives:	
To evaluate the efficacy of atezolizumab as neoadjuvant treatment for Stage IB, II, IIIA, and selected IIIB NSCLC	Pathological response in PD-L1-positive and PD-L1 negative groups
To evaluate response to atezolizumab in patients with PD-L1-positive vs. PD-L1-negative tumors	Investigator-assessed response rate per RECIST v1.1
Exploratory Efficacy Objectives:	

Safety Objectives:	
To evaluate the safety and tolerability of atezolizumab as neoadjuvant or adjuvant treatment	Incidence of adverse events, with severity per NCI CTCAE v4.0
Exploratory Biomarker Objectives:	

CNB = core needle biopsy; [REDACTED]

NCI CTCAE v4.0 = National Cancer Institute

Common Terminology Criteria for Adverse Events, version 4.0; [REDACTED]

[REDACTED] NSCLC= non-small cell lung cancer; [REDACTED]

ORR= objective response rate; [REDACTED] PD-L1= programmed death ligand 1; RECIST v1.1 = Response Evaluation Criteria in Solid Tumors, version 1.1; WES = whole exome sequencing.

3. STUDY DESIGN

3.1 Overview of Study Design

This is a Phase II, open-label, single-arm study, designed to evaluate the efficacy of atezolizumab as a neoadjuvant therapy in patients with Stage IB, II, IIIA, or selected IIIB NSCLC scheduled for curative-intent resection by evaluating response to treatment. In addition, the safety of atezolizumab will be evaluated in this treatment setting. [REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

Patients will be closely monitored for safety and tolerability throughout the study. Safety assessments will include collection and monitoring of adverse events and laboratory abnormalities graded per the National Cancer Institute Common Terminology Criteria for Adverse Events, Version 4.0 (NCI CTCAE v4.0). Laboratory safety assessments will include the regular monitoring of hematology and blood chemistry. [REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

The Neoadjuvant Atezolizumab Therapy Phase will end when the last data point required for primary efficacy analysis (i.e., pathology evaluation at time of surgery) is obtained. This is expected to occur approximately 30 days after the last patient's surgery.

[REDACTED]

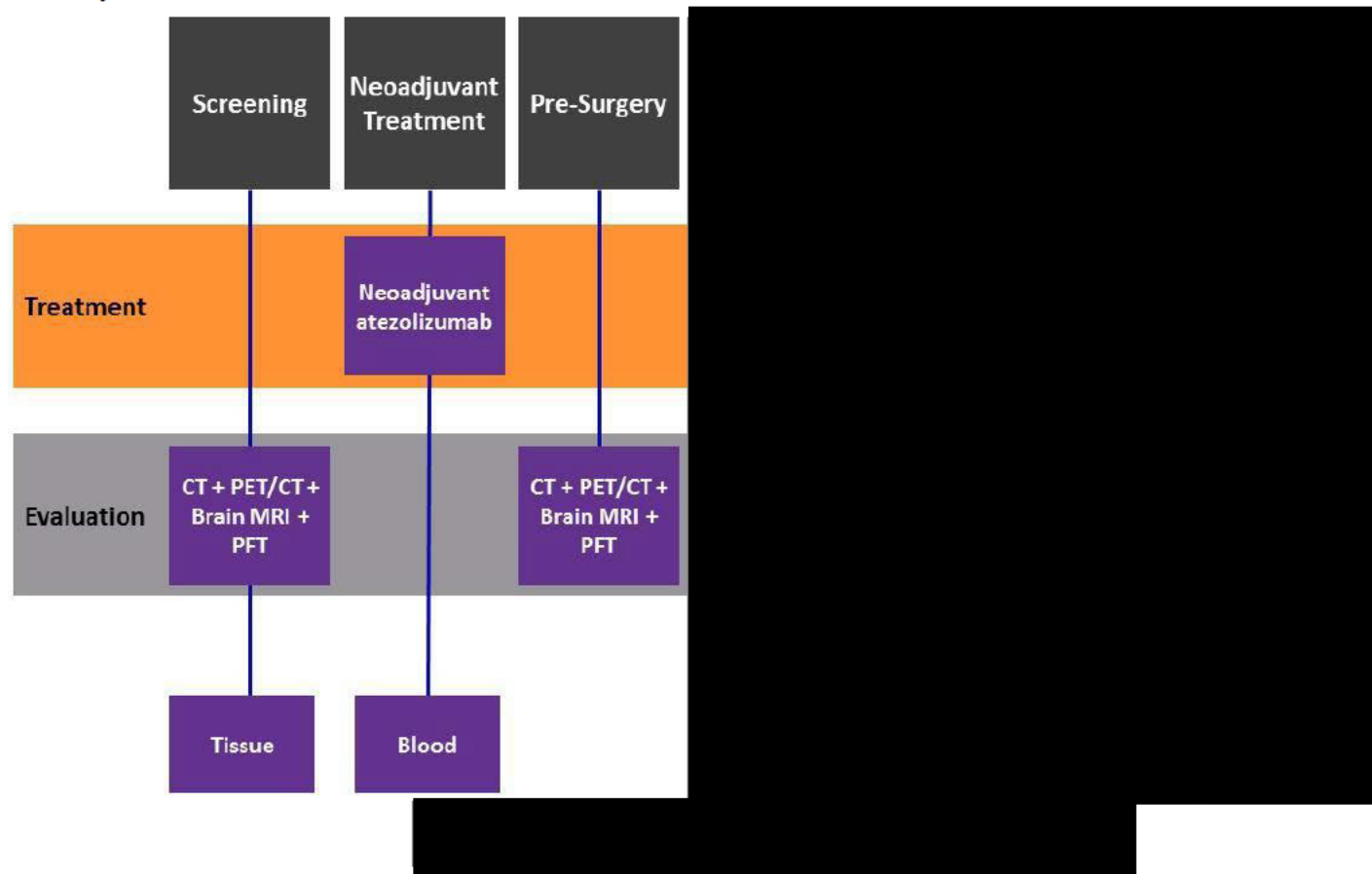
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[REDACTED]

[REDACTED]

[REDACTED]

The study schema is illustrated in Figure 1, and a schedule of assessments is presented in Section 4 of this statistical analysis plan.



a Refer to Appendix 2 in the protocol , Timing of Surveillance Assessments.



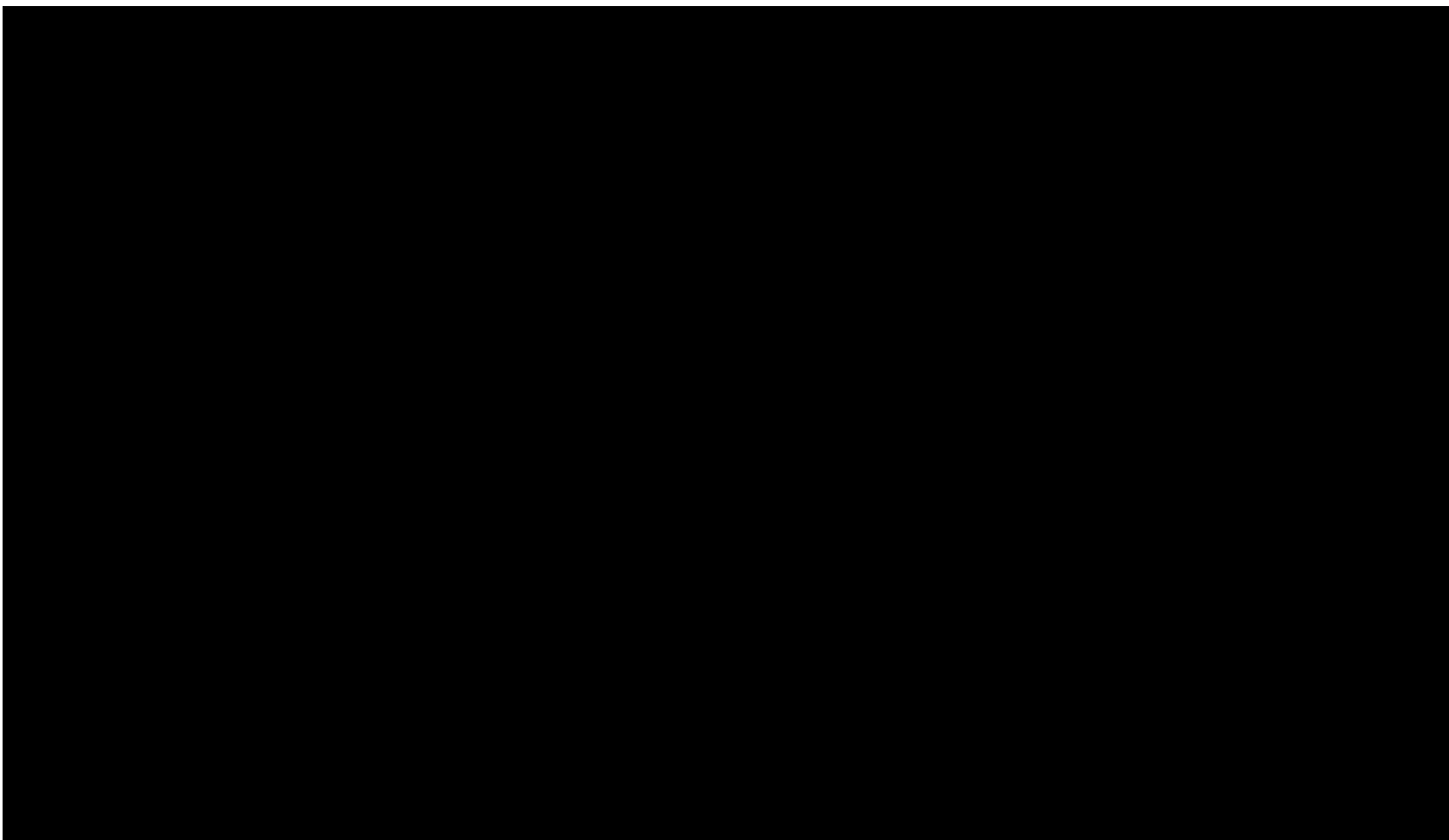
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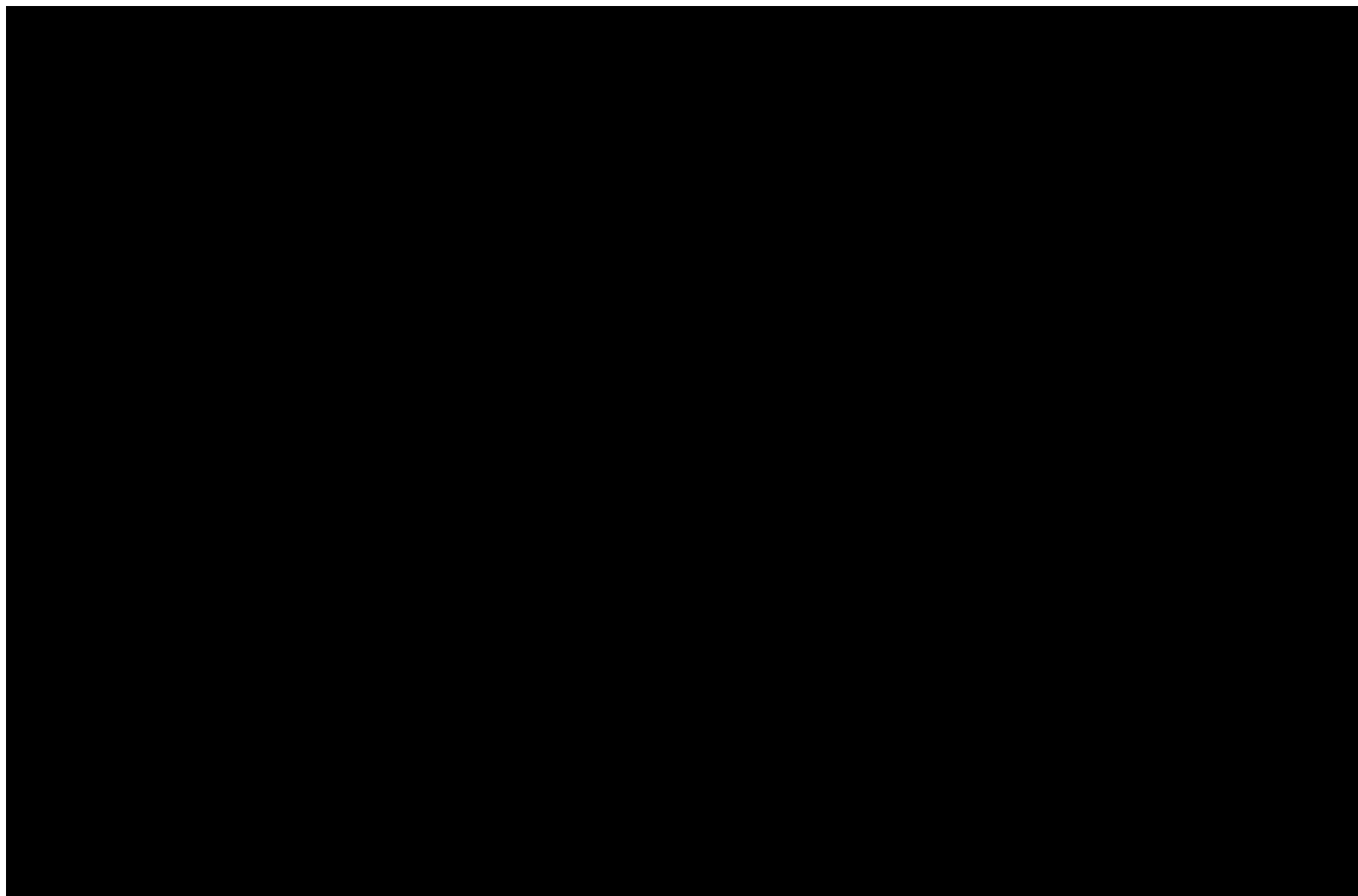
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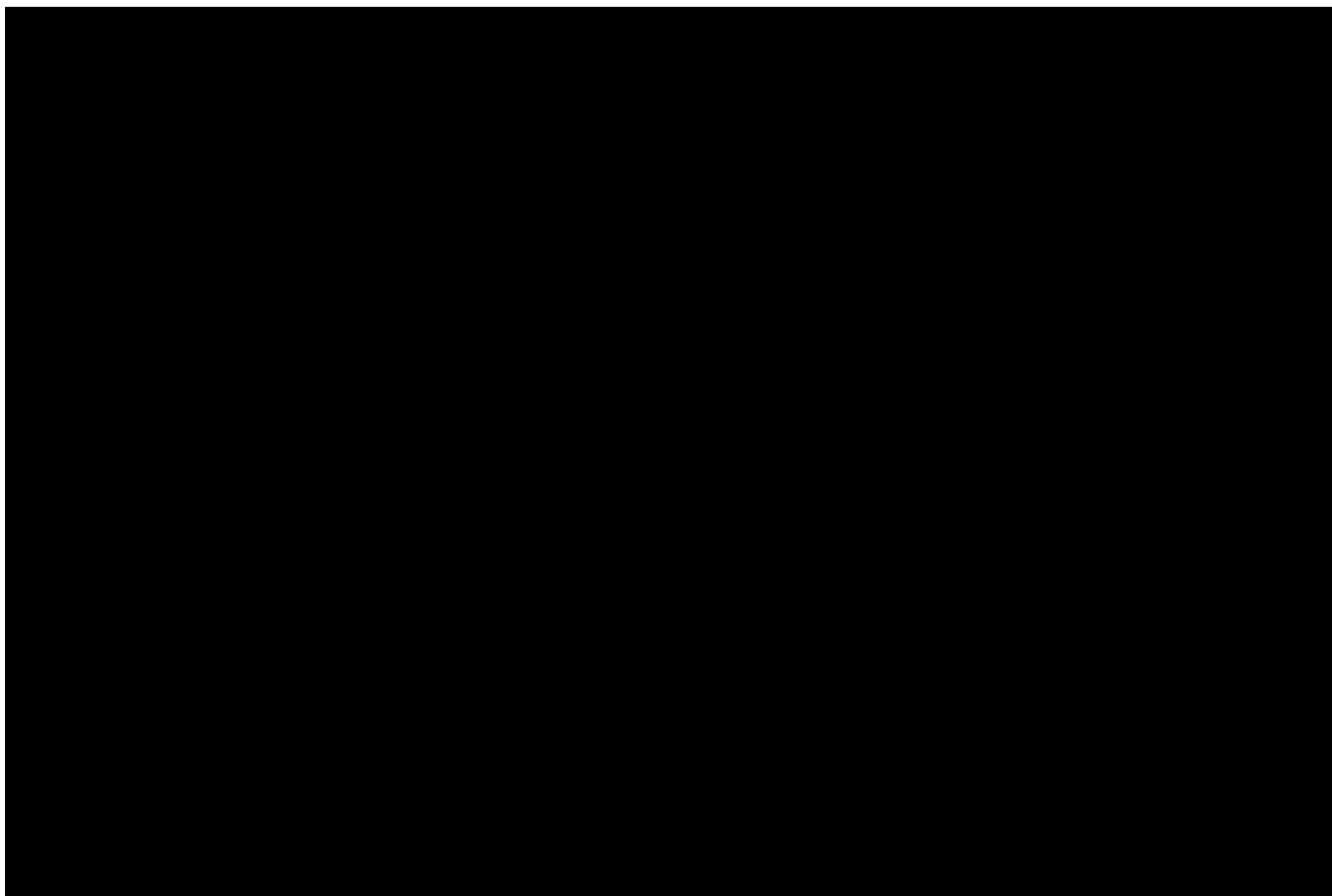
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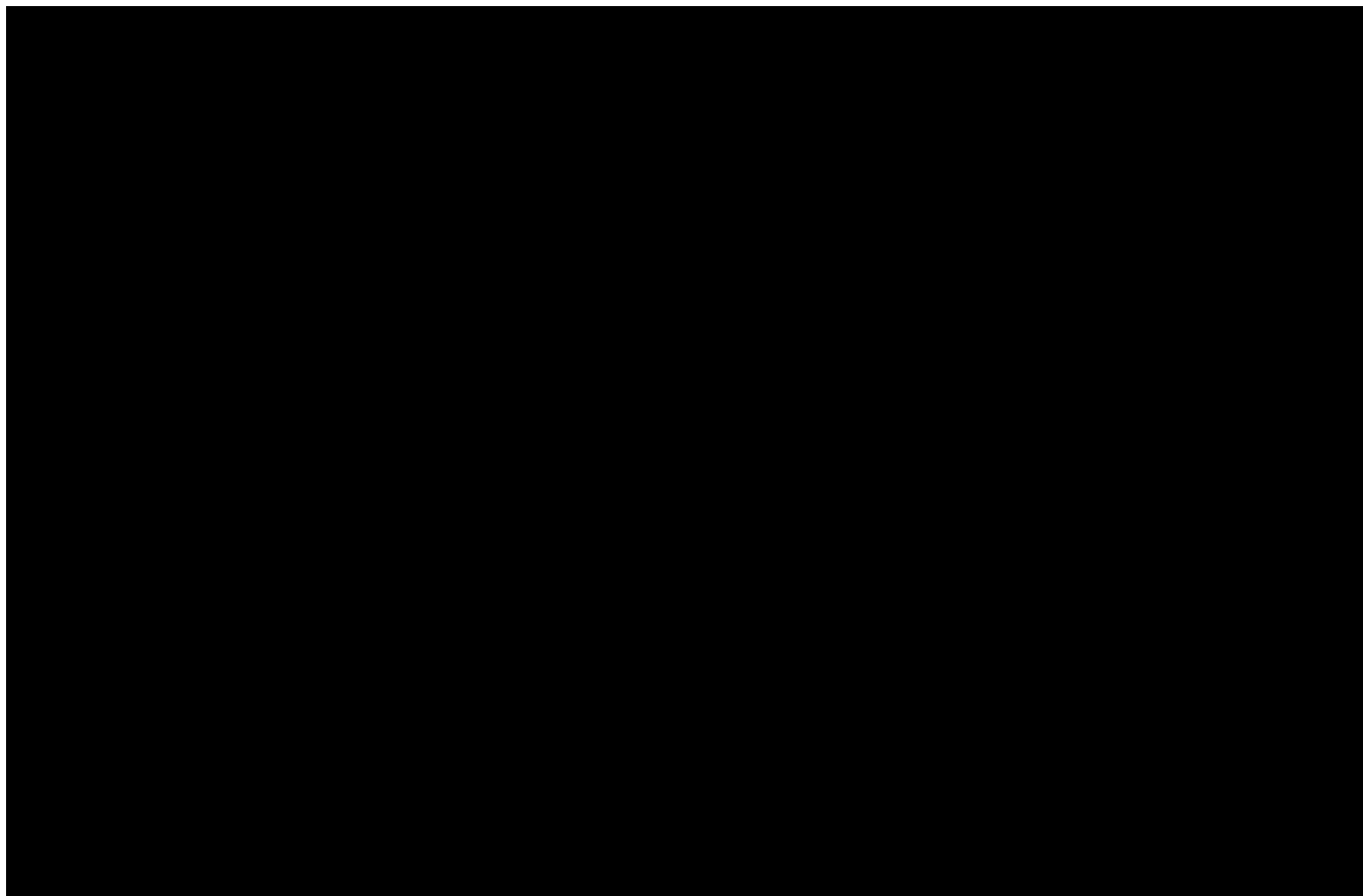
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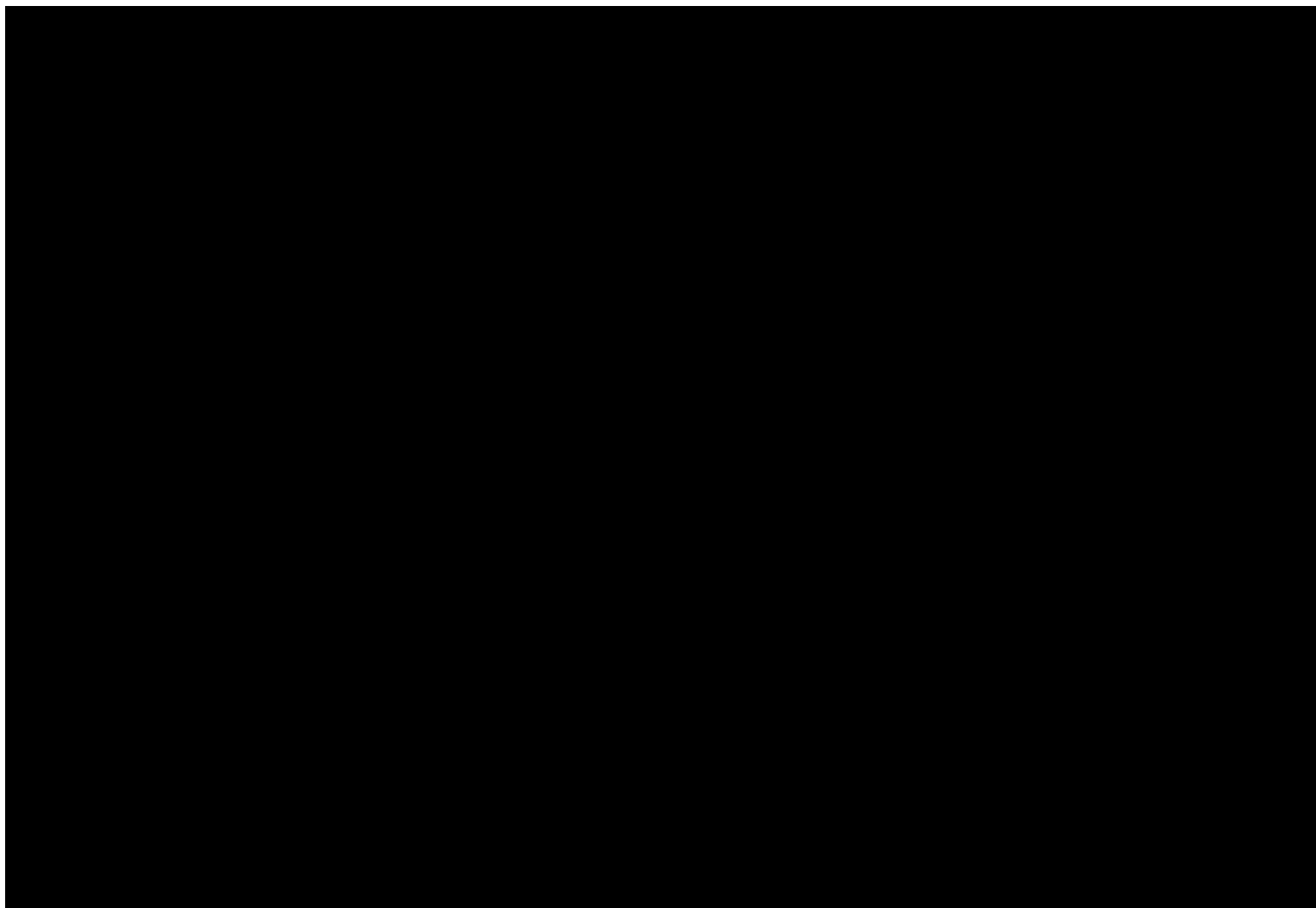
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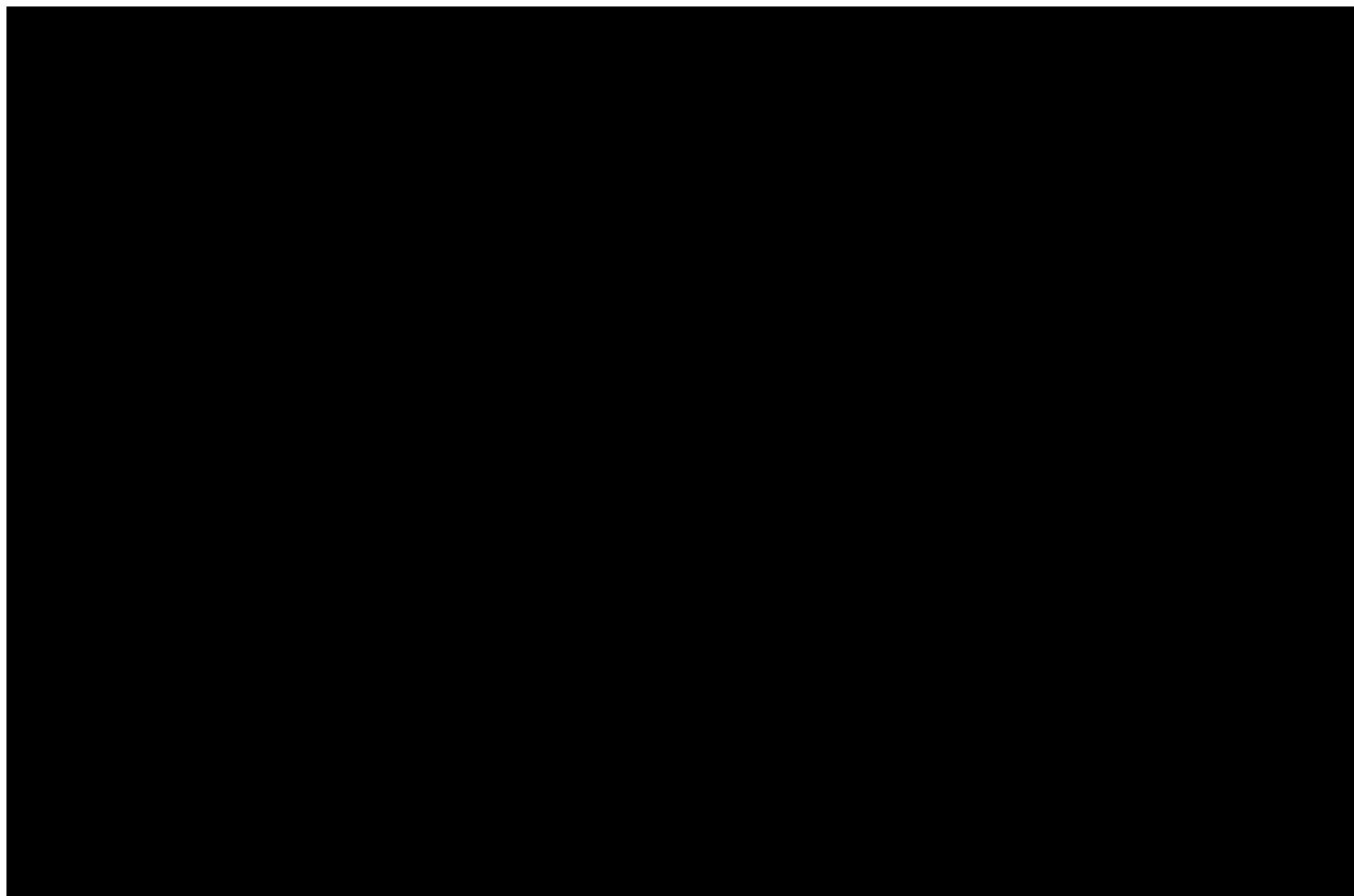
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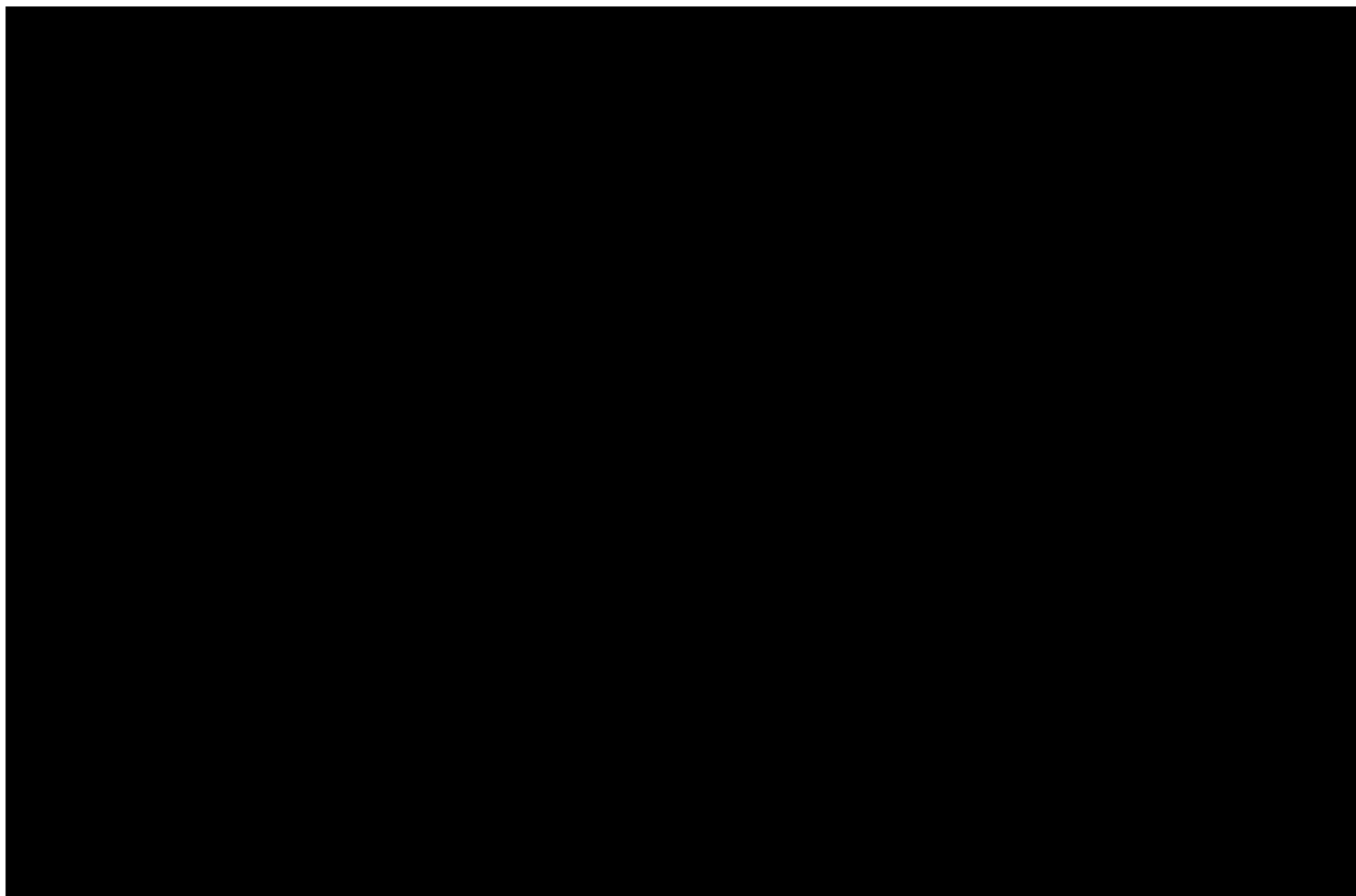
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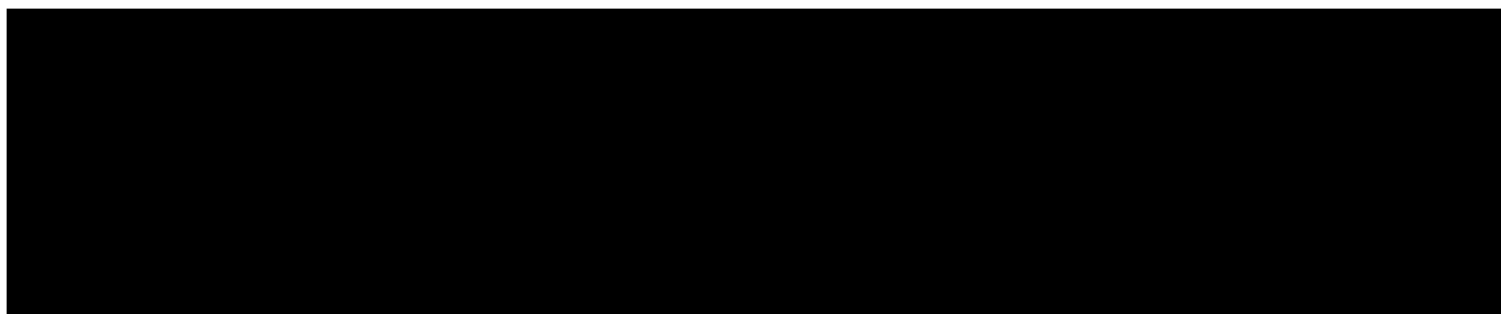
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5. ANALYSIS POPULATIONS

Data will be summarized using the following analysis populations.

- All Enrolled Subjects: This analysis population will include all enrolled subjects, regardless of whether they received any amount of study drug. The all enrolled subjects population will be used for the summarization of subject disposition.
- Safety Population: This analysis population will include all enrolled subjects who have received any dose of study drug. The safety population will be used for the analysis of safety measures.
- Efficacy Population: This analysis population will include all enrolled patients who have received at least one dose of the study drug and who do not have EGFR or ALK mutant tumors.
- Primary Efficacy Population: This analysis population will include all efficacy population subjects, excluding patients who have not received surgery after neoadjuvant treatment with Atezolizumab, and excluding subjects who have not had a complete resection. This population will be used for primary efficacy analysis. For the primary efficacy population, if major pathological response is unable to be assessed, then it will remain undetermined and the subject will be removed from this population.

6. STATISTICAL METHODOLOGY

6.1 Statistical and Analytical Issues

6.1.1 Statistical Methods

Continuous data will be summarized by descriptive statistics, including mean, standard deviation, median, minimum, and maximum. Categorical data will be summarized by the number and percentage of subjects. Percentages will be based on non-missing data unless specified otherwise. Specific details regarding statistical testing will be found in Section 6.3 of this SAP.

Baseline values will be the latest assessment prior to the start of the first cycle of study drug. The same baseline will be used for each study phase.

Statistical analyses will be carried out using SAS® software version 9.3 or higher (Copyright, SAS Institute Inc. SAS and all other SAS Institute Inc. product or service names are registered trademarks or trademarks of SAS Institute, Inc., Cary, North Carolina, USA).

6.1.2 Handling of Dropouts and Missing Data

Adverse Events

- Adverse events for which the relation to study drug is not reported will be assumed related to study drug.

- Adverse events for which the NCI CTCAE grade is not reported will not have a grade imputed and will be summarized as “No Grade”.

1. [REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]

■ [REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]

■ [REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]

[illegible]

■ [REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]

Efficacy Analysis

Handling of missing data for efficacy analysis will be discussed in the respective sections below in this SAP.

6.1.3 Pooling of Investigative Sites

Data from all sites will be pooled for summarization and analyses.

[illegible]

[REDACTED]

[REDACTED]

[REDACTED]

6.2 Subject Characteristics

6.2.1 Subject Disposition

Neoadjuvant Atezolizumab Therapy Phase

Calculating the percentages based on the number of subjects who received neoadjuvant atezolizumab treatment, summarize the following:

- The number of subjects who receive neoadjuvant atezolizumab (treated)
- The number and percentage of subjects who had surgery
- The number and percentage of treated subjects who completed Neoadjuvant Atezolizumab treatment
- The number and percentage of subjects who do not complete Neoadjuvant Atezolizumab treatment
- The number and percentage of subjects who do not complete Neoadjuvant Atezolizumab treatment by reason

[REDACTED]

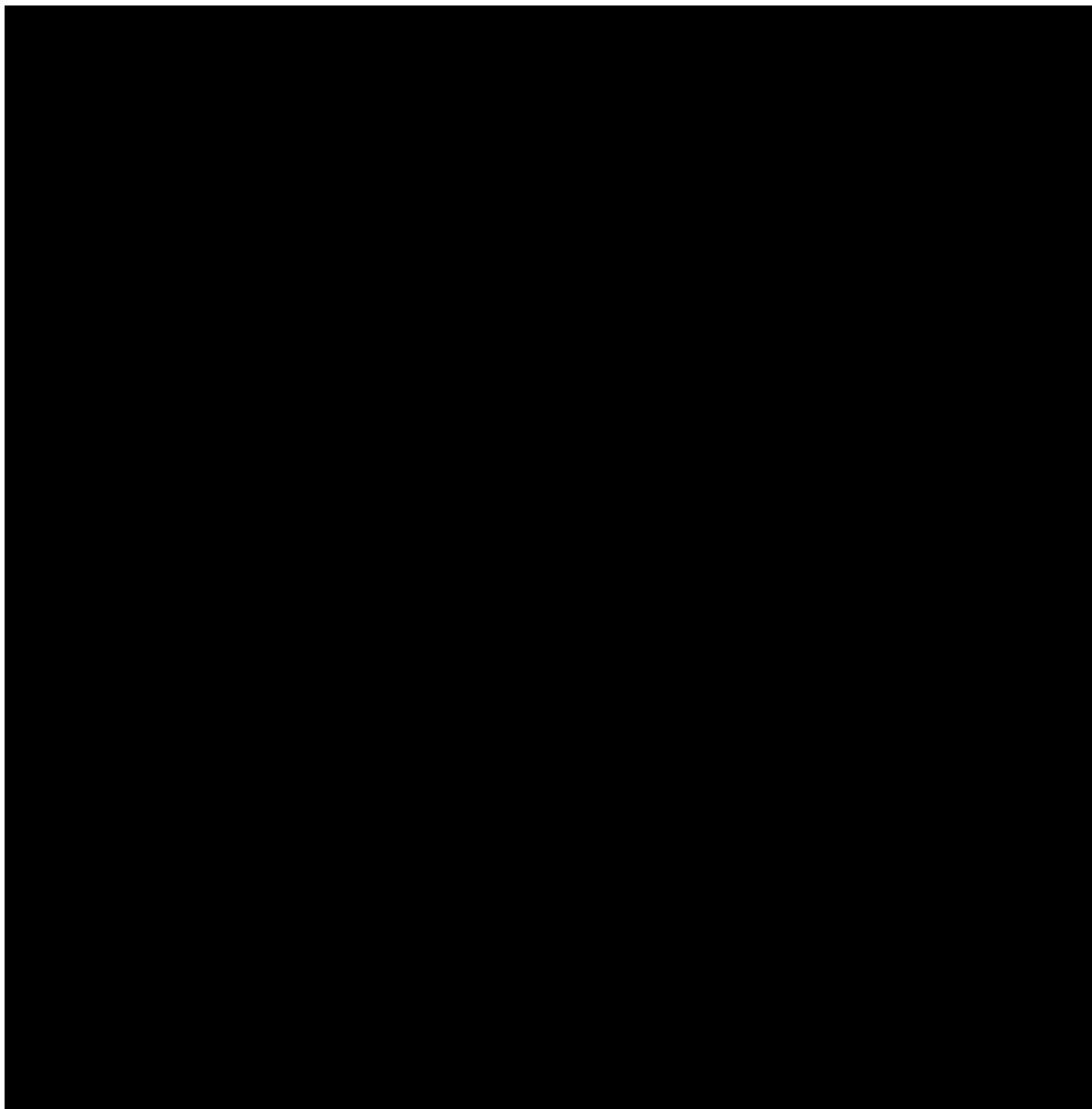
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- [REDACTED]
- [REDACTED]
- [REDACTED]
- [REDACTED]

Study Discontinuation

Calculating the percentages based on the number of subjects enrolled, summarize the following:

- The number and percentage of subjects who completed the study,
- The number and percentage of subjects who discontinued the study
- The number and percentage of subjects who discontinued the study by reason

[illegible]



6.2.4 Treatment Exposure and Compliance

Neoadjuvant Atezolizumab Therapy Phase

Treatment exposure and compliance will be examined in the Neoadjuvant Atezolizumab Therapy Phase of the study by summarizing overall the number of cycles initiated, infusion cycle intervention status (yes, no), and reason for infusion intervention (infusion related reaction [IRR], adverse event not considered IRR, other) for the safety population.

[REDACTED]

6.2.5 Prior and Concomitant Medications and Therapies

All reported medications and therapies, whether determined to be prior or concomitant uses, will be coded using the Genentech, Inc. Drug Thesaurus. Levels of summarization will include drug class and standardized term.

Concomitant medications are defined as non-study medications with a stop date on or after the first dose of study drug, and a start date on or before the date of the subject's last visit in the study. Medications that started prior to the first dose of study drug and are ongoing will be classified as concomitant. Medications that have a stop date prior to the first dose of study drug will be flagged as 'prior'.

At each level of summarization (drug class and standardized term), a subject will be counted only once for each medication he/she has within that level. Prior and concomitant medications will be summarized in separate tables for each study phase separately. The percentage of safety population subjects having had at least one prior or concomitant medication at each level will be calculated.

Data listings will be provided for all prior and concomitant medications.

6.2.6 Medical Histories

Medical conditions will be coded to system organ classes and preferred terms using Medical Dictionary for Regulatory Activities version 19.1 or higher if available.

The number and percentage of safety population subjects who have any medical history and any medical history within the system organ class and preferred term levels will be summarized.

6.2.7 Surgeries and Procedures

Surgeries and procedures will be coded to standardized terminology.

The number and percentage of safety population subjects with any surgery or procedure, any surgery, and any procedure will be summarized.

6.3 Efficacy Analysis

6.3.1 Primary Efficacy Analysis

The number of subjects with major pathologic response (n) and the major pathologic response rate ($n/N \times 100$) will be summarized for the primary efficacy population (N). A one sample exact binomial test will be performed to test the null hypothesis that the major pathologic response rate is 5%, versus the alternative hypothesis is the major pathologic response rate is $> 5\%$. If the null hypothesis is rejected then this is evidence that the response rate exceeds 5% when atezolizumab is given before resection. The one-sided 95% confidence interval (CI) for major pathologic response rate will be reported.



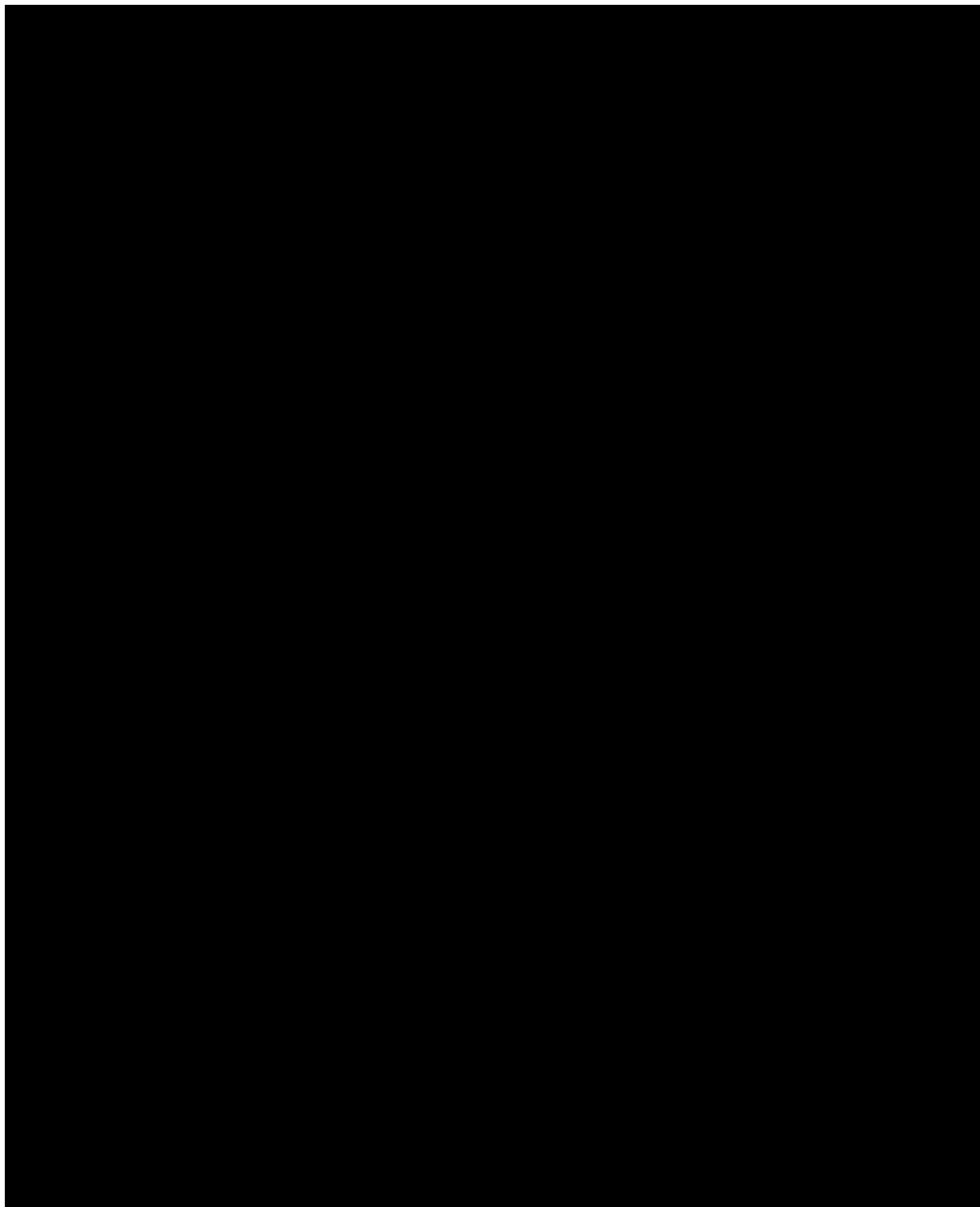
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6.4 Safety Analysis

6.4.1 Adverse Events (AE)

All reported adverse events terms will be coded to a system organ class (SOC) and preferred term (PT) using the Medical Dictionary for Regulatory Activities version 19.1 or higher if available.

Treatment related adverse events are those adverse events flagged as suspected to be caused by Atezolizumab.

Neoadjuvant Atezolizumab Therapy Phase

For the Neoadjuvant Atezolizumab Therapy Phase of the study, an adverse event will be any event that starts following the start of the first dose of study drug for those subjects not continuing in the Surveillance/Survival Follow-up Phase of the study, and through the day immediately preceding the start date of the first adjuvant cycle for subjects continuing in the Surveillance/Survival Follow-up Phase of the study.

If the onset date and/or resolution date of an adverse event is incomplete (for example, only the month and year are available), the event will be assumed to start after the first dose of study drug, as long as the available information does not conclusively indicate that the event emerged prior to the start date of the first cycle of study drug administration.

An overview of adverse events will present the number and percentage of safety population subjects who have at least one AE, at least one Grade 3 or higher AE, at least one treatment related AE, at least one serious AE, at least one AE of special interest, at least one AE leading to study drug withdrawal, at least one AE leading to death, and at least one treatment related AE leading to death, overall (subjects counted only once if any AE is experienced) and by highest NCI CTCAE grade (subjects counted only once at the highest grade level experienced).

The number and percentage of safety population subjects with AEs will be summarized by SOC and PT in total, and by highest NCI CTCAE grade. At each summary level, a subject will be counted only once if he/she experiences at least one AE, and when summarizing by highest NCI CTCAE grade, a subject will be counted only once at the highest grade level experienced. AE summary tables will be sorted by descending occurrence rate according to the SOC, and then by descending occurrence rate within PT. The following types of AE summaries will be prepared:

- All AE
- Grade ≥ 3 AE
- Onset date prior to date of surgery (Neoadjuvant Atezolizumab Therapy Phase only)
- Onset date on or after date of surgery, for safety population with surgery date (Neoadjuvant Atezolizumab Therapy Phase only)
- Treatment related AE
- Serious AE
- AE of special interest
- AE leading to death
- Treatment related AE leading to death
- AEs leading to study drug withdrawal

All adverse events will be listed including the AEs which start before the first dose of study drug, and additional separate listings will be created for AEs leading to death, serious AEs, and AEs leading to study drug withdrawal.

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

6.4.2 Laboratory Parameters

Serum chemistry and hematology parameters will be converted to a standard set of units to be agreed upon with the sponsor.

Neoadjuvant Atezolizumab Therapy Phase

Converted laboratory parameter values will be summarized descriptively (n, mean, standard deviation, median, minimum, and maximum) for safety population subjects at each scheduled data collection time point. Change from baseline (the latest value collected prior to first dose) to each scheduled data collection time point will also be summarized descriptively for safety population subjects.

Shift tables will present a cross tabulation of baseline results categories (values falling in low, normal, high categories relative to normal ranges, or values being associated with grades) versus result categories at each follow up visit. The number and percentage of safety population subjects falling within each cross tabulation cell will be presented; the denominator for each percentage will be the number of safety population subjects with a baseline result and a follow-up result at the given visit for the given parameter.

6.4.3 Vital Signs

Vital signs will be summarized descriptively (n, mean, standard deviation, median, minimum, and maximum) for safety population subjects at each scheduled data collection time point. Raw and change from baseline vital sign values, including systolic and diastolic blood pressure, heart rate, height, weight, body temperature for both oral and auricular, and respiratory rate, will be summarized using descriptive statistics at baseline and each assessment time point at post-dose visits for each of the two study phases.

6.4.4 Physical Examination

Complete physical examination at screening and limited physical examination at each scheduled data collection time point will be listed for both phases of the study.

6.4.5 Pulmonary Function Test

Raw and change from baseline for pulmonary function test will be summarized descriptively (n, mean, standard deviation, median, minimum, and maximum) and listed at each scheduled data collection time point for the Neoadjuvant Atezolizumab Therapy Phase.

6.4.6 ECOG Performance Status

Number and percentage for each ECOG performance status will be summarized and listed at each scheduled data collection time point for both phases of the study.

6.4.7 Analyses of Other Safety Assessments

The data for coagulation (aPTT and INR) tests, Pregnancy test, urinalysis, thyroid function test (TSH, Free T3 and Free T4), C-reactive protein, viral serology will be listed.

6.5 Interim Analysis

The study will continue during the planned interim analyses.

6.5.1 Planned Interim Analysis

6.5.1.1 *Planned Interim Safety Analysis*

After the first 30 patients complete protocol-specified atezolizumab treatment and undergo the planned surgical resection of their tumor, all safety population subjects at that time will be evaluated for tolerability and safety of the neoadjuvant regimen. If, at the time of the safety interim analysis, three or more patients have experienced Grade 5 adverse events related to protocol treatment, then the study may be discontinued.

Outputs from this SAP which will facilitate the planned interim safety analysis are flagged in Section 9 of this SAP.

6.5.1.2 *Planned Interim Efficacy Analysis*

The study includes one interim analysis to assess futility after 90 patients are assessed for MPR. The non-binding futility interim boundary satisfies conditional power at a cut-off of 0.30.

The analysis for the primary endpoint will be conducted and assessed against the futility boundary. If there are 7 or fewer responders out of 90 patients, then the conditional power of the study drops below 0.30, and enrollment may be terminated.

Outputs from this SAP which will facilitate the planned interim efficacy analysis are flagged in Section 9 of this SAP.

6.5.2 Optional Interim Analysis

Given the hypothesis-generating nature of this study, the sponsor may choose to conduct additional interim efficacy analyses. The decision to conduct an optional interim analysis, and the timing of the analysis, will be documented in the sponsor's trial master file prior to the conduct of the interim analysis. The interim analysis will be performed and interpreted by sponsor study team personnel.

6.6 Changes to Methods Planned in the Protocol

There is no change in the SAP which is deviate from protocol.

7. TABLES, LISTINGS, AND FIGURES

Programmed output will have the following features.

- Courier New Font, 9 point.
- All outputs will be landscape orientation with 1.00 inch left, right, and bottom margins, and a 1.25-inch top margin.
- General header format – analysis population will not appear on listings:

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[output type name] [output number]
[output title]
([analysis population name])

- Source program name and program run date will appear in output footer.
- Unless specified otherwise, minimum and maximum will be displayed with the same precision as recorded data, mean and median will be displayed with one decimal place more than the recorded data, and standard deviation will be displayed with two decimal places more than the recorded data.

Table, listing, and figure shells can be found in the attached document, Genentech_ML39236_SAP_shells_client_v_2_0.docx. A table of contents for outputs to be provided is in Table 3.

Table 3 Tables, Listings, and Figures to Be Produced

Interim Analysis Flag	Output Type and Number	Output Title	Output Population
S, E, P	Table 14.1.1.1	Subject Disposition, Neoadjuvant Atezolizumab Therapy Phase	All Enrolled Subjects
P	Table 14.1.1.4	Subject Discontinuation	All Enrolled Subjects
S, E, P	Table 14.1.2	Demographics and Baseline Characteristics, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.1.3	General Medical History and Baseline Conditions	Safety Population
	Table 14.1.4	Surgeries and Procedures	Safety Population
S, E, P	Table 14.1.5	Tobacco Use History, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.1.6	Non-metastatic Lung Cancer History, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.1.7.1	Lung Cancer Molecular Profile, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.1.7.2	Derived Lung Cancer Molecular Profile, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S	Table 14.1.8	Screening Ventilation / Perfusion Scan Results	Safety Population
P	Table 14.1.9	Major Protocol Deviations	Safety Population
S, E, P	Table 14.1.10.1	Study Drug Exposure, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, P	Table 14.1.11.1	Prior Medications	Safety Population
S, P	Table 14.1.11.2	Concomitant Medications, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
E, P	Table 14.2.1	Primary Efficacy Analysis for Major Pathologic Response Rate	Primary Efficacy Population

Interim Analysis Flag	Output Type and Number	Output Title	Output Population
S, E, P	Table 14.3.1.1.1	Overview of Adverse Events by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.2.1	Adverse Events by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.2a	Adverse Events by Highest NCI CTCAE Grade, Both Study Phases	Safety Population
S, E, P	Table 14.3.1.2b	Adverse Events With Onset Prior to Date of Surgery by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.2c	Adverse Events With Onset On or After Date of Surgery by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population With Surgery
S, E, P	Table 14.3.1.3.1	Grade ≥ 3 Adverse Events by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population

Interim Analysis Flag	Output Type and	Output	Output
S, E, P	Table 14.3.1.4.1	Treatment Related Adverse Events by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.4a	Treatment Related Adverse Events With Onset Prior to Date of Surgery by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.4b	Treatment Related Adverse Events With Onset On or After Date of Surgery by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population With Surgery
S, E, P	Table 14.3.1.5.1	Serious Adverse Events by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.5a	Treatment Related Serious Adverse Events by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.5b	Serious Adverse Events With Onset Prior to Date of Surgery by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.5c	Serious Adverse Events With Onset On or After Date of Surgery by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population With Surgery
S, E, P	Table 14.3.1.6.1	Adverse Events of Special Interest by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.7.1	Adverse Events Leading to Study Drug Withdrawal by Highest NCI CTCAE Grade, Neoadjuvant Atezolizumab Therapy Phase	Safety Population

Interim Analysis Flag	Output Type and Number	Output Title	Output Population
P	Table 14.3.1.8.1	Adverse Events Leading to Death, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Table 14.3.1.9.1	Treatment Related Adverse Events Leading to Death, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Listing 14.3.2.1.1	Listing of Adverse Events Leading to Death, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Listing 14.3.2.2.1	Listing of Serious Adverse Events, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Listing 14.3.2.3.1	Listing of Adverse Events Leading to Study Drug Withdrawal, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, P	Table 14.3.3.1.1	Serum Chemistry Values Over Time, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, P	Table 14.3.3.2.1	Hematology Values Over Time, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
	Table 14.3.3.3.1	Shift Table of Serum Chemistry Values, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
	Table 14.3.3.4.1	Shift Table of Hematology Values, Neoadjuvant Atezolizumab Therapy Phase	Safety Population

Interim Analysis Flag	Output Type and Number	Output Title	Output Population
P	Table 14.3.4.1	Vital Sign Values Over Time, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
	Table 14.3.5.1	Pulmonary Function Test Over Time, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
P	Table 14.3.6.1	Number and Percentage for ECOG, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
P	Listing 16.2.1.1	Listing of Subject Disposition, Neoadjuvant Atezolizumab Therapy Phase	All Enrolled Subjects
P	Listing 16.2.2.1	Listing of Baseline Demographics	All Enrolled Subjects
P	Listing 16.2.2.2	Listing of Baseline Characteristics	Safety Population
	Listing 16.2.3.1	Listing of Tobacco Use History, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Listing 16.2.3.2	Listing of Non-metastatic Lung Cancer History, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
S, E, P	Listing 16.2.3.3	Listing of Lung Cancer Molecular Profile, Neoadjuvant Atezolizumab Therapy Phase	Safety Population
	Listing 16.2.3.4	Listing of Screening Ventilation / Perfusion Scan	Safety Population
	Listing 16.2.4.1	Listing of General Medical History and Baseline Conditions	Safety Population
	Listing 16.2.4.2	Listing of Surgeries and Procedures	Safety Population
	Listing 16.2.4.3	Listing of Prior and Concomitant Medications	Safety Population
P	Listing 16.2.4.4	Protocol Deviation	Safety Population
S, E, P	Listing 16.2.5	Listing of Study Drug Administration	Safety Population
E, P	Listing 16.2.6.1	Pathological Response	Efficacy Population

Interim Analysis Flag	Output Type and Number	Output Title	Output Population
S, E, P	Listing 16.2.7.1	Listing of Adverse Events	Safety Population
S, E, P	Listing 16.2.7.2	Listing of Serious Adverse Event Details	Safety Population
	Listing 16.2.8.1	Listing of Serum Chemistry	Safety Population
	Listing 16.2.8.2	Listing of Hematology	Safety Population
	Listing 16.2.8.3	Listing of Vital Signs	Safety Population
	Listing 16.2.8.4	Complete Physical Examination	Safety Population
	Listing 16.2.8.5	Pulmonary Function Tests	Safety Population
	Listing 16.2.8.6	ECOG Performance Status	Safety Population
	Listing 16.2.8.7	Other Safety Assessments	Safety Population
P	Figure 1.1	Forest Plot of Major Pathological Responders	Efficacy Population



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Interim Analysis Flag	Output Type and Number	Output Title	Output Population

Note: Interim analysis flags are S = Safety Interim Analysis and E = Efficacy Interim Analysis. P = Primary Analysis.

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